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Case Number: 26CHCV01171 Hearing Date: September 10, 2026 Dept: 1

DEMURRER

Moving Party: Defendants Kaiser Foundation Health

Plan, Inc., Kaiser Foundation Hospitals, and Kaiser Permanente Insurance Co.

(Maynard Nexsen LLP)

Responding

Party: Plaintiff Henry Mayo Newhall Hospital (King & Spalding

LLP)

Ruling: Demurrer sustained on

the First Cause of Action as to KFH without leave, on the Second Cause of Action as to KPIC without leave, on the Third Cause of Action as to KFH with 30 days leave to amend, and on the Fourth Cause of Action as to KFH without leave and as to KPIC and KFHP with 30 days leave to amend, and otherwise overruled.

This is an implied contract and Knox-Keene Act suit by plaintiff Henry Mayo Newhall Hospital (Plaintiff) against defendants Kaiser Foundation Health Plan, Inc. (KFHP), Kaiser Foundation Hospitals (KFH), and Kaiser Permanente Insurance Co. (KPIC) (collectively, Defendants). Plaintiff alleges that it provided emergency medical services and post-stabilization care to patients for which Defendants were obligated to pay, but that Defendants' payments were not the amounts required by law. Defendants demur to Plaintiff's operative Second Amended Complaint (the SAC).

LEGAL STANDARDS

A demurrer tests the sufficiency of a complaint as a matter

of law and raises only questions of law. (Code Civ. Proc. § 589; Schmidt v. Foundation Health (1995) 35 Cal.App.4th 1702, 1706.) ¿When considering demurrers, courts read the allegations liberally and in context.¿ (Taylor v. City of Los Angeles Dept. of Water and Power¿(2006) 144 Cal.App.4th 1216, 1228.)¿ In a demurrer proceeding, the defects must be apparent on the face of the pleading or by proper judicial notice. (Code Civ. Proc. § 430.30(a).)¿ A demurrer tests the pleadings alone and not the evidence or other extrinsic matters. (SKF Farms v. Superior Court (1984) 153 Cal. App. 3d 902, 905.)¿ Therefore, it lies only where the defects appear on the face of the pleading or are judicially noticed. (Ibid.)¿ The only issue involved in a demurrer hearing is whether the complaint, as it stands, unconnected with extraneous matters, states a cause of action.¿ (Hahn v. Mirda (2007) 147 Cal.App.4th 740, 747.)

“On demurrer the allegations of the complaint are assumed to be true. A demurrer is simply not the appropriate procedure for determining the truth of disputed facts.” (Ramsden v. Western Union (1977) 71 Cal.App.3d 873, 879.) “Less particularity is required when it appears that defendant has superior knowledge of the facts, so long as the pleading gives notice of the issues sufficient to enable preparation of a defense.” (Okun v. Superior Court (1981) 29 Cal.3d 442, 458 (Okun).) “If the complaint states a cause of action under any theory, regardless of the title under which the factual basis for relief is stated, that aspect of the complaint is good against a demurrer.” (Quelimane Co. v. Stewart Title Guaranty Co. (1998) 19 Cal.4th 26, 38.) “[A] plaintiff is required only to set forth the essential facts of his case with reasonable precision and with particularity sufficient to acquaint a defendant with the nature, source and extent of his cause of action.” (Youngman v. Nevada Irrigation District (1969) 70 Cal.2d 240, 245 (Youngman); see also Doheny Park Terrace Homeowners Assn., Inc. v. Truck Ins. Exchange (2005) 132 Cal.App.4th 1076, 1099 (Doheny Park Terrace).)

REQUESTS FOR JUDICIAL NOTICE

Defendants

request judicial notice of the following items:

1. The

California Department of Health Care Access and Information (“HCAI”) license information for Kaiser Foundation Hospitals. A true and correct copy of a PDF printout from the HCAI’s website showing that Kaiser Foundations Hospitals operates pursuant to numerous medical facility licenses, publicly available at <https://lfis.hcai.ca.gov/Licensee/Detail/1453>;

2. The

CDI Company Profile of Kaiser Permanente Insurance Company, publicly available at https://interactive.web.insurance.ca.gov/apex_extprd/

[f?p=144:6:15603304843687::NO:RP,6:P6_COMPANY_ID,P6_NAIC:70891,60053&cs=1UKIXL3jJZLXH-Lgi2LRFHcNrWyA;](https://interactive.web.insurance.ca.gov/apex_extprd/?p=144:6:15603304843687::NO:RP,6:P6_COMPANY_ID,P6_NAIC:70891,60053&cs=1UKIXL3jJZLXH-Lgi2LRFHcNrWyA;)

3. The

DMHC Health Plan Directory showing an absence of KPIC and KFH, but listing KFHP as a California health plan, publicly available at <https://wpso.dmhc.ca.gov/>

[Dashboard/SearchHealthPlan.aspx](https://wpso.dmhc.ca.gov/Dashboard/SearchHealthPlan.aspx);

4. The

CDI Certificate of Authority for Kaiser Permanente Insurance Company; and

5. The

CDI Certificate of Compliance for Kaiser Permanente Insurance Company.

All of these are matters capable of immediate and accurate determination by resort to sources of reasonably indisputable accuracy, and the request is therefore GRANTED in full.

Plaintiff requests

judicial notice of various matters:

1. The

November 10, 2025 Memorandum from the Second District, Division Five, of the California Court of Appeal denying the California Association of Health Plans’ request for the publication of the October 20, 2025 decision in Prime Healthcare Centinela, LLC v. United Healthcare Insurance Co., Case No. B334746. This is a court record and subject to judicial notice under Evid. Code § 452(d).

2. The

Register of Actions of California Supreme Court Case No. S293947, in the matter of the publication request for the October 20, 2025 Court of Appeal decision in Prime Healthcare Centinela, LLC v. United Healthcare Insurance Co. showing that the request was denied. This is a court record and subject to judicial notice under Evid. Code § 452(d).

3. A.B.

1203, as chaptered on September 30, 2008, compared to previous version of A.B.

1203, as amended in Senate on June 30, 2008, available online at

https://leginfo.legislature.ca.gov/faces/billVersionsCompareClient.xhtml?bill_id=200720080AB1203&cversion=20070AB120394AMD. This is a proposition not reasonably subject

to dispute and capable of immediate and accurate determination by resort to a source of reasonably indisputable accuracy, and subject to judicial notice under Evid. Code § 452(h).

4. The

September 15, 2023 response from the Department of Managed Health Care (DMHC) to a California Public Records Act request, producing Kaiser Permanente's February 3, 2022 Letter to California Regulators Regarding No Surprises Act Preemption of California State Law. This is an official act of an executive department of the State of California and subject to judicial notice under Evid. Code § 452(c).

5. The

March 21, 2022 "APL 22-0111 – No Surprises Act (NSA) Guidance" from the Department of Managed Health Care, available at

[www.dmhc.ca.gov/Portals/0/Docs/OPL/APL%2022-011%20-%20No%20Surprises%20Act%20\(NSA\)%20Guidance%20\(3_21_22\).pdf?ver=2022-03-21-171002-097](http://www.dmhc.ca.gov/Portals/0/Docs/OPL/APL%2022-011%20-%20No%20Surprises%20Act%20(NSA)%20Guidance%20(3_21_22).pdf?ver=2022-03-21-171002-097). This is an official act of an executive

department of the State of California and subject to judicial notice under Evid. Code § 452(c).

Plaintiff's

request is therefore GRANTED in full.

On reply,

Defendants request judicial notice of a letter from the Center for Medicare and Medicaid Services to various California officials concerning the No Surprises Act. This is an official act of an executive department of the United States and subject to judicial notice under

Evid. Code § 452(c). The request is GRANTED as to this item. Defendants also request judicial notice of a ruling by another department of this Court on a demurrer and motion to strike by Kaiser Foundation Health Plan, Inc. The ruling of another court of equal jurisdiction does not bind this one, and it is further improper to present this three-year-old purported authority on reply. The request is DENIED as to the trial court ruling.

DISCUSSION

I.

ORDER TO SHOW CAUSE AS TO MAYNARD NEXSEN LLP'S
VIOLATION OF RULE 8.1115 BY DELIBERATELY CITING AN UNPUBLISHED CASE

First, the Court must address Defendants' citation of Prime Healthcare Centinela, LLC v. United Healthcare Ins. Co. (Ct. App. Oct. 20, 2025) 2025 WL 2950428 (nonpub. opn.) at page 5 of their demurrer. There is no question it is an unpublished case, which cannot be cited except under narrow circumstances not present here. (See Cal. Rules of Court, rule 8.1115(a), (b).) Defendants clearly recognize that this case is unpublished and not citable, as they state that "Kaiser does not cite this unpublished opinion as binding legal authority, but simply to provide a complete record to the Court concerning the history of this legal issue." This is therefore not a situation where an attorney innocently but mistakenly relies on an unpublished case believing it to be published, but rather a deliberate citation of an unpublished case. There is absolutely no basis for Defendants' citation of this case, which is a clear violation of Rule 8.1115.

For these reasons, the Court issues an order pursuant to Cal. Rules of Court, rule 2.30 for Defendants' counsel Maynard Nexsen LLP to show cause why they should not be ordered to pay \$1,000 in sanctions to the Court for their violation of Cal. Rules of Court, rule 8.1115. The hearing on the OSC will be set at the upcoming hearing on Defendants' demurrer.

II.

CLAIMS AGAINST KAISER PERMANENTE INSURANCE
COMPANY

A.

Quantum Meruit Under Knox-Keene Act (COA 1)

Defendants contend that KPIC is not a health care service plan regulated by the Department of Managed Health Care (DMHC) subject to the Knox-Keene Act, but rather an insurer regulated by the California Department of Insurance. Plaintiff's SAC does so allege. (See SAC, ¶ 3.) However, Plaintiff's First Cause of Action is not asserted against KFIC. (See FAC, p. 10.) This is therefore a moot point.

B.

Quantum Meruit Under Insurance Code (COA 2)

"The requisite elements of quantum meruit are (1) the plaintiff acted pursuant to 'an explicit or implicit request for the services' by the defendant, and (2) the services conferred a benefit on the defendant." (Port Medical Wellness, Inc. v. Connecticut General Life Ins. Co. (2018) 24 Cal.App.5th 153, 180 (Port Medical).)

While contending that no express request is required, Plaintiff argues that a request may be implied in law under the circumstances even if there has been no conduct on the insurer's part. The question here is whether there was a request by KFIC, implied in law or fact, for Plaintiff to render emergency services to its insureds.

In this cause of action, Plaintiff alleges that the Insurance Code requires insurers such as KPIC to cover emergency services, including with non-contracting providers. (SAC, ¶ 49, citing Ins. Code §§ 10112.27(a)(1) [requiring individual policies to cover emergency services], 10112.281(a)(6) [requiring large group health insurance policies to cover emergency health care services].) As Defendants note, those statutes simply require coverage for emergency services under the insurance policy, not any payments to providers such as Plaintiff. Defendants contrast the Insurance Code with the Knox-Keene Act in the Health and Safety Code, which specifically requires reimbursement (and which KPIC is concededly not subject to). (See Health & Saf. Code § 1371.4(b) ["A health care service plan, or its contracting medical providers, shall reimburse providers for emergency services

and care provided to its enrollees..."].)

Plaintiff cites Ins. Code § 10112.7, which likewise requires coverage of emergency services even as to “a nonparticipating health care provider with or without prior authorization” or out-of-network. (See *Id.* at (a)(3).) Next, Plaintiff cites Ins. Code §§ 10123.13 and 10123.147, which require prompt notice of contested and denied claims and prompt reimbursement of completed claims, and Ins. Code § 10123.137(b), which requires insurers to provide a dispute resolution mechanism that “is accessible to noncontracting providers for the purpose of resolving billing and claims disputes.” Plaintiff then cites Cal. Code Regs., tit. 10, § 2562.06(k), a regulation pertaining to coverage for mental health conditions and substance abuse disorders (not emergency services), which provides that an insurer shall not

[f]ail or refuse to directly reimburse an out-of-network health care provider or facility, in compliance with Insurance Code section 10123.13 or 10123.147, as applicable, for covered health care benefits delivered by the provider or facility when requiring an assignment is prohibited by law, or pursuant to a valid assignment of benefits made by an insured to the provider or facility, irrespective of any conflicting anti-assignment provisions included in an insured's coverage document.

(*Ibid.*)

Plaintiff cites *Bell v. Blue Cross of California* (2005) 131 Cal.App.4th 211 (*Bell*), in which the Court of Appeal held that an emergency services provider had an implied-in-law right to recover the reasonable value of his services (that is, quantum meruit) from a health care service plan based on the requirement for such plans to reimburse providers under Health & Saf. Code § 1371.4(b):

We reject Blue Cross's contention that Dr. Bell has no implied-in-law right to recover for the reasonable value of his services. “He who takes the benefit must bear the burden” (Civ.Code, § 3521), and he who has “performed the duty of another by supplying a third person with necessities, although acting without the other's knowledge or consent, is entitled to restitution from the other therefore if [¶] (a) he acted unofficiously and with intent to charge therefor, and [¶] (b) the things or services supplied were immediately necessary to prevent serious bodily harm to or suffering by such person.” (Rest., Restitution, § 114 (1937),

quoted in *California Emergency Physicians Medical Group v. PacifiCare of California*, supra, 111 Cal.App.4th at p. 1137, fn. 3, 4 Cal.Rptr.3d 583.) Dr. Bell's quantum meruit claim is sufficient for pleading purposes and thus is not subject to demurrer.

(Bell, supra, 131 Cal.App.4th at 221.)

It is notable that none of Plaintiff's cases involve insurance companies, but rather health care service plans. *Nissanoff v. UnitedHealthCare Ins. Co.* (2024) 108 Cal.App.5th Supp. 1 (*Nissanoff*) makes clear that the regulatory scheme applicable to health care service plans does not apply to CDI-regulated insurers like KPIC, providing:

The law is clear. Managed health care service plans are subject to the Knox-Keene Act and regulated by the DMHC. Life and disability insurer and insurance companies are subject to the Insurance Code and regulated by the CDI. [Citation.] Thus, the only scenario in which defendant would be subject to the jurisdiction of the Knox-Keene Act would be if it were directly providing the health care service through entity-owned or contracting health facilities and providers. (See Health & Saf. Code, § 1343, subd. (e)(1).)

(Id. at 10.)

Plaintiff contends correctly that *Nissanoff* is not binding, as it is a decision of the Appellate Division of this Court and not of the Court of Appeal, but not that it is wrong. Plaintiff then states that *Nissanoff* "did not address...whether quantum meruit exists independently of a statutory framework."

While quantum meruit is plainly not statutory, it still requires a promise implied in law or fact for the services rendered. (*Port Medical*, supra, 24 Cal.App.5th at 180.) As to managed health care service plans, that promise is implied in law by Health & Saf. Code § 1371.4(b)'s unambiguous requirement that those plans pay providers for emergency services rendered directly to their enrollees. (See Health & Saf. Code § 1371.4(b); Bell, supra, 131 Cal.App.4th 211, 215 ["A health care service plan, or its contracting medical providers, shall reimburse providers for emergency services and care provided to its enrollees..."].) Plaintiff's statutory authority, meanwhile, pertains to coverage and claims processing by insurers,

and contains no express language regarding the obligation to reimburse or pay providers. The only express language Plaintiff presents is in a regulation pertaining to mental health and substance abuse treatment “when requiring an assignment is prohibited by law or pursuant to a valid assignment made by an insured to the provider” and is inapplicable here. (See Cal. Code Regs., tit. 10, § 2562.06(k).)

Defendants also submit that the Court of Appeal has held that prompt payment provisions like the ones on which Plaintiff relies (including Ins. Code §§ 10123.13 and 10123.147) do not necessarily create a quantum meruit claim. (See *Pomona Valley Hospital Medical Center v. Kaiser Foundation Health Plan, Inc.* (2026) 119 Cal.App.5th 43, 64-68 [recognizing that prompt payment regulations enacted under the Knox-Keene Act did not apply to the valuation required for quantum meruit claim].)

Plaintiff cites Health & Safety Code § 1317, which is not part of the Knox-Keene Act, for the proposition that there is some legal obligation for insurers to pay providers for emergency care rendered to their insureds. The pertinent provision reads:

(d) Emergency services and care shall be rendered without first questioning the patient or any other person as to his or her ability to pay therefor. However, the patient or his or her legally responsible relative or guardian shall execute an agreement to pay therefor *or otherwise supply insurance or credit information promptly after the services are rendered. *

(Health & Saf. Code § 1317(d), emphasis added.) As Defendants point out, however, this is an obligation for patients or their representatives to supply insurance information to emergency medical services providers, not an obligation for insurers to pay those providers. It in no way resembles the clear statutory obligation set out in Health & Saf. Code § 1371.4(b). Plaintiff’s contention that this indicates a legislative intent that insurers pay providers directly for emergency services simply does not follow, as no such intent is apparent from the statute.

Given the foregoing, the Court concludes there is no request

implied in law for the provision of emergency services to KPIC insureds and thus this cause of action fails. It is therefore unnecessary for the Court to address Defendants' contention that the federal No Surprises Act (NSA) preempts state law claims for quantum meruit because the NSA governs payment for emergency services rendered by noncontracted providers where there is no "specified state law" providing for a determination of the total amount payable. It is likewise unnecessary to address Defendants' contention that this cause of action is uncertain. Plaintiff's theory of liability under the Insurance Code is legally deficient, so leave to amend would be futile and will be denied.

C.

Breach of Contract Implied in Law for
Post-Stabilization Services (COA 3)

This cause of action is not asserted against KPIC.

D.

Breach of Contract Implied in Fact for
Post-Stabilization Services (COA 4)

A contract implied in fact differs from an express contract in that its "existence and terms are manifested by conduct." (McGough v. University of San Francisco (1989) 214 Cal.App.3d 1577, 1584, citing Civ. Code § 1621.) "The distinction between express and implied in fact contracts relates only to the manifestation of assent; both types are based upon the expressed or apparent intention of the parties. The true implied contract ... consists of obligations arising from a mutual agreement and intent to promise where the agreement and promise have not been expressed in words. [Citation.]" (Ibid, emphasis in original, internal quotation marks omitted.) In other words, a contract implied in fact still requires assent, but rather than being communicated through words or through a signature on a written agreement, assent is communicated by conduct.

The case of Pacific Bay Recovery, Inc. v. California Physicians' Services, Inc. (2017) 12 Cal.App.5th 200 (Pacific Bay) is instructive. There, the defendant was a health care service plan under the Knox-Keene Act, while the

plaintiff was a medical provider. (Id. at 203-204.) When one of the defendant's insureds was admitted to the plaintiff's treatment program for 31 days, the plaintiff submitted several invoices to the defendant. (Id. at 204.) The defendant paid for only 6 of the 31 days. (Ibid.) The plaintiff sued for, among other things, breach of implied contract. (Ibid.) Affirming the sustaining of a demurrer, the Court of Appeal rejected the plaintiff's argument that the insurer's practice of paying only a portion of the invoices showed the existence of an implied contract:

The fact that Blue Shield only paid for six of the 31 days of treatment undermines Pacific Bay's claim that the parties ever agreed to the same contractual terms. By way of Blue Shield's conduct, it appears that it believed it was to pay for only six days. In contrast, Pacific Bay argues that it was to be paid for the entire length of treatment. Thus, the allegations in the FAC, based on Blue Shield's payment of some of the invoices, does not exhibit any mutual intent as to the essential terms of the implied contract.

(Id. at 216.) The plaintiff had also alleged that the defendant had agreed to an implied contract by providing authorizations for the patients' care:

Pacific Bay contends that it properly alleged the elements of an implied contract in paragraphs 13 and 14 of the FAC. In paragraph 13, Pacific Bay alleged that it "contacted Blue Shield to obtain prior authorization, precertification and consent to render treatment and perform procedures upon" the subscriber. "At all relevant times, [Pacific Bay] was advised by representatives of Blue Shield that the [subscriber] was insured, covered, and eligible for coverage under the respective Plan or Policy for the services to be rendered by [Pacific Bay], at facilities operated by [Pacific Bay] and that [Pacific Bay] would be paid for performance of the procedures, care, and/or treatment rendered by Blue Shield." In paragraph 14, Pacific Bay further averred that it "was led to believe that it would be paid a portion or percentage of its total billed charges, which charges correlated with usual, reasonable and customary charges."

These allegations lack the specific facts required for us to determine there was any meeting of the minds between the parties. At best, Pacific Bay's allegations show that Blue Shield admitted that

the subscriber was covered under one of its health plans and that it would pay something for Pacific Bay's treatment of the subscriber. What type of treatment or the extent of treatment is not described. In addition, it does not appear the parties reached any sort of agreement as to the rate Blue Shield would pay Pacific Bay. Indeed, Pacific Bay alleged it was led to believe Blue Shield would pay "a portion or percentage of its total billed charges, which charges correlated with usual, reasonable and customary charges." Blue Shield did pay a portion of the billed charges, but Pacific Bay argues it was not enough. However, we cannot say Blue Shield's payments breached any implied contract because there is no indication in the FAC what exactly Blue Shield agreed to pay.

(Pacific Bay, *supra*, 12 Cal.App.5th at 216.) Here, Plaintiff alleges:

62. Implied-in-fact contracts are created between Henry Mayo and Kaiser which requires Kaiser to pay Henry Mayo its charges whenever Kaiser authorizes Henry Mayo to provide post-stabilization services to its members but fails to negotiate a discounted rate.

63. In authorizing Henry Mayo to provide the post-stabilization services, Kaiser does not request that Henry Mayo discount its charges for the post-stabilization services that Kaiser requests Henry Mayo provide to Kaiser's members, nor does Henry Mayo agree to discount its charges.

64. At the time that Kaiser authorizes Henry Mayo to provide the post-stabilization services to its members, Kaiser is aware of Henry Mayo's charges for its services. Henry Mayo's charges are publicly available on Henry Mayo's website, as required by law, and Kaiser had received, and continues to receive, Henry Mayo's bills which contained the rates Henry Mayo charges for its services, as well as changes to Henry Mayo's charges over time. If Kaiser did not want to pay Henry Mayo's billed charges for the post-stabilization services, Kaiser can transfer its members to Kaiser or other hospitals or negotiate a payment rate with Henry Mayo.

65. Henry Mayo performs all of the obligations required to be performed under the implied-in-fact contracts with Kaiser.

66. Kaiser has breached these implied-in-fact contracts by failing to pay Henry Mayo its charges for the post-stabilization services that Henry Mayo provided to Kaiser's members, and continues to breach such contracts to the extent that it continues to fail to pay Henry Mayo its charges for such services.

(FAC, ¶¶ 62-66.)

In essence, Plaintiff alleges that by providing authorizations for treatment of their insureds or members with knowledge of Plaintiff's publicized rates, Defendants impliedly agreed to pay those rates. California law requires Plaintiff to plead the agreed-upon rate, which under these allegations can only be Plaintiff's posted rates. (See *Allied Anesthesia Medical Group, Inc. v. Inland Empire Health Plan* (2022) 80 Cal.App.5th 794, 810 ["A claim for breach of an implied-in-fact contract requires plaintiffs to plead the agreed-upon rate"].) Pacific Bay expressly rejected the theory that an insurer implicitly agrees to pay posted rates by authorizing treatment. (*Pacific Bay*, supra, 12 Cal.App.5th at 216.) This cause of action therefore fails.

III.

CLAIMS AGAINST KAISER FOUNDATION HOSPITALS

A.

Quantum Meruit Under Knox-Keene Act (COA 1)

Defendants contend that this cause of action fails against KFH because it is not a health service plan, but rather a hospital system that holds licenses to operate medical facilities. (See Defendants' Request for Judicial Notice (RJN), Ex. A, Ex. C.) Plaintiff does not dispute that KFH is not a Knox-Keene Act health plan but argues that KFH is liable as an alter ego of Kaiser Foundation Health Plan (KFHP), which is. Alter ego is not pled in the complaint, and Plaintiff cannot invoke it now. Moreover, the Court is disinclined to grant leave to amend to allege alter ego in this context. It is so commonly known within this jurisdiction as to be subject to judicial notice that Kaiser Foundation Health Plan is one of the largest, if not the largest managed health care plan in California. It beggars belief to imagine that an inequitable result could obtain if Plaintiff's Knox-Keene Act remedies are limited to KFHP rather than KFH. The

Court's tentative is therefore to sustain the demurrer to this cause of action without leave as to KFH. It is unnecessary to consider Defendants' other challenges to this cause of action as to KFH.

B.

Breach of Contract Implied in Law for
Post-Stabilization Services (COA 3)

Defendants contend that this cause of action fails because, like the First Cause of Action, it is premised on KFH being a managed health care plan when it is not. Plaintiff alleges:

57. Implied-in-law contracts are created between Henry Mayo and KFH which requires KFH shall pay charges when KFH (a) fails to respond to Henry Mayo's requests for authorization to provide post-stabilization care within 30 minutes; Health & Saf. Code § 1262.8(d)(2); or (b) responds that it would assume management of the patient's care by prompt transfer, but fails to promptly transfer the member to another hospital. Health & Saf. Code § 1262.8(d)(3).

(FAC, ¶ 57.) These provisions of the Health and Safety Code are indeed applicable to "health care service plans" which, as discussed, KFH is not.

(See Defendants' RJN, Ex. A, Ex. C.)

Plaintiff identifies no other source of an agreement implied in law for KFH to pay for post-stabilization services Plaintiff rendered to patients. As discussed above, Plaintiff's alter ego theory is unpled and seemingly without any plausible support. The Court is therefore inclined to likewise sustain the demurrer to this cause of action without leave to amend as to KFH.

C.

Breach of Contract Implied in Fact for
Post-Stabilization Services (COA 4)

As discussed above, there are legal problems with Plaintiff's theory of liability under this cause of action, because Pacific Bay provides that an authorization to provide treatment is not an implicit agreement to pay posted rates. (Pacific Bay, *supra*, 12 Cal.App.5th

at 216.) There is a further issue with this cause of action as to KFHP, because it is undisputed that it is not a health insurer or managed health care plan. The Court is therefore inclined to sustain the demurrer to this cause of action.

IV.

CLAIMS AGAINST KAISER FOUNDATION HEALTH PLAN

A.

Breach of Contract Implied in Fact for
Post-Stabilization Services (COA 4)

For the reasons discussed above as to both KFIC and KFHP, Plaintiff has not stated sufficient facts to constitute a cause of action for implied contract. (See *Pacific Bay*, supra, 12 Cal.App.5th at 216.) The Court will sustain the demurrer.

V.

UNCERTAINTY

For the reasons discussed above, many of Plaintiff's causes of action are subject to demurrer on substantive grounds, and it is unnecessary to discuss uncertainty in detail.

However, the Court does nonetheless agree that Plaintiff's treatment of the defendants as interchangeable renders the FAC uncertain. Defendants are distinct types of entities: KFHP is a managed health care plan, KFHP is a hospital system, and KPIC is an insurance company. Each has a particular legal form and nature and is subject to different, often non-overlapping regulatory schemes. Thus, while uncertainty is not dispositive of any of the causes of action above because of the other grounds discussed, it is very important that Plaintiff make clear in any amended complaint what conduct is alleged against which entity, which entity has what legal duties, and so on.

CONCLUSION

For the foregoing reasons, the Court rules as follows.

.

The demurrer to the First Cause of Action for Quantum Meruit under the Knox-Keene Act is SUSTAINED as to KFH without leave to amend and OVERRULED AS MOOT as to KPIC;

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The demurrer to the Second Cause of Action for Quantum Meruit under the Insurance Code is SUSTAINED as to KPIC without leave to amend;

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The demurrer to the Third Cause of Action for Breach of Contract Implied in Law is SUSTAINED as to KFH with 30 days leave to amend; and

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The demurrer to the Fourth Cause of Action for Breach of Contract Implied in Fact is SUSTAINED as to KFH without leave to amend and as to KPIC and KFHP with 30 days leave to amend.

Except as expressly sustained herein, the demurrers are OVERRULED. Defendants to give notice.